

RESEARCH REPORT

Promotora and Community Navigation Models for Breast Cancer in California, 2014-2023

An implementation-focused review of screening access, diagnostic follow-up, and Latina survivorship support

Evidence window January 2014 through December 2023; foundational navigation evidence and current program data included

Central conclusion

Community health workers and promotoras add the greatest value when they do more than deliver information. Their distinctive contribution is trusted, language-concordant accompaniment that converts a recommendation into completed screening, diagnostic resolution, and survivorship support.

Executive summary

Promotoras and community health workers are widely used in breast cancer outreach, but the label can describe very different interventions. Some programs ask community workers to distribute education or reminders. Others authorize them to schedule appointments, identify financial and transportation barriers, communicate with clinical teams, follow abnormal results, and support survivors after treatment. The literature reviewed for this report indicates that the second model is more consistent with the mechanisms through which navigation reduces disparities [4-7].

California offers an especially strong setting for this model because the state combines large Latino and immigrant populations, substantial rural and agricultural regions, safety-net care, and specialized cancer services concentrated in metropolitan areas. Community workers can bridge these settings, but only when their role is integrated with clinical referral systems and supported by stable financing.

The evidence includes national screening reviews, patient-navigation research, federal safety-net program data, and California-developed community interventions. The Nuevo Amanecer program, developed with urban and rural Latina breast cancer survivors in California, demonstrated that a culturally and linguistically tailored, peer-delivered intervention could improve quality of life and reduce depressive and somatic symptoms. Its relevance extends beyond survivorship: it shows that community expertise is not simply a recruitment tool, but part of the intervention itself [11].

Implementation distinction

A promotora program is not defined by who delivers the message. It is defined by the functions the program enables: trust-building, barrier assessment, scheduling, accompaniment, referral coordination, and follow-through across the care continuum.

Key findings

- Navigation consistently addresses barriers that educational materials cannot: transportation, appointment coordination, language access, insurance problems, fear, and fragmented referrals [6,7].
- The federal NBCCEDP has served more than 6.6 million women and provided more than 17.0 million breast and cervical screening examinations since 1991, demonstrating the scale of organized safety-net delivery [3].
- During 2020-2021, NBCCEDP reached 13.5% of the estimated 2.6 million U.S. women eligible for program-supported breast screening, showing that need remains much larger than funded capacity [3].
- California's Nuevo Amanecer intervention supports the use of trained peers for culturally tailored breast cancer survivorship care, with reported improvements in quality of life and reductions in depressive and somatic symptoms [11].
- Programs should distinguish outreach completion from care completion. The core outcomes are screening completed, abnormal finding resolved, treatment linked, and survivorship needs addressed.
- Workforce continuity, supervision, referral authority, and data access determine whether a community-worker model can function as navigation rather than education alone.

1. Background

Breast cancer disparities are produced at multiple points. Some people are never reached by routine screening systems. Others receive a recommendation but cannot obtain an appointment. Still others complete a mammogram but encounter delays in diagnostic imaging, biopsy, or treatment. After treatment, low-income and limited-English-proficient survivors may have fewer resources for symptom management, psychosocial support, and survivorship planning. Promotoras and community health workers can operate across these boundaries because they are positioned between community life and formal care.

The patient-navigation model was developed to address the “discovery-delivery disconnect”: the gap between identifying a possible cancer and delivering timely, complete care. Navigators identify barriers and work with patients and systems to resolve them. Community health workers may perform many of the same functions, with an additional emphasis on shared community experience, cultural knowledge, and trust [7].

For California, the most useful question is therefore not whether community workers are effective in the abstract. It is which functions, organizational supports, and referral relationships allow them to improve breast cancer outcomes across diverse communities.

2. Methods

This implementation-focused narrative review covered evidence published or operational during 2014-2023, supplemented by foundational navigation literature and current federal program reporting. Sources included systematic reviews, peer-reviewed California community interventions, federal safety-net program documentation, and implementation-science frameworks.

Evidence was organized by intervention function rather than job title: community outreach and trust-building; screening navigation; abnormal-result follow-up; treatment and survivorship navigation; and organizational infrastructure. Numerical values were extracted directly from sources. No combined effect estimate was calculated.

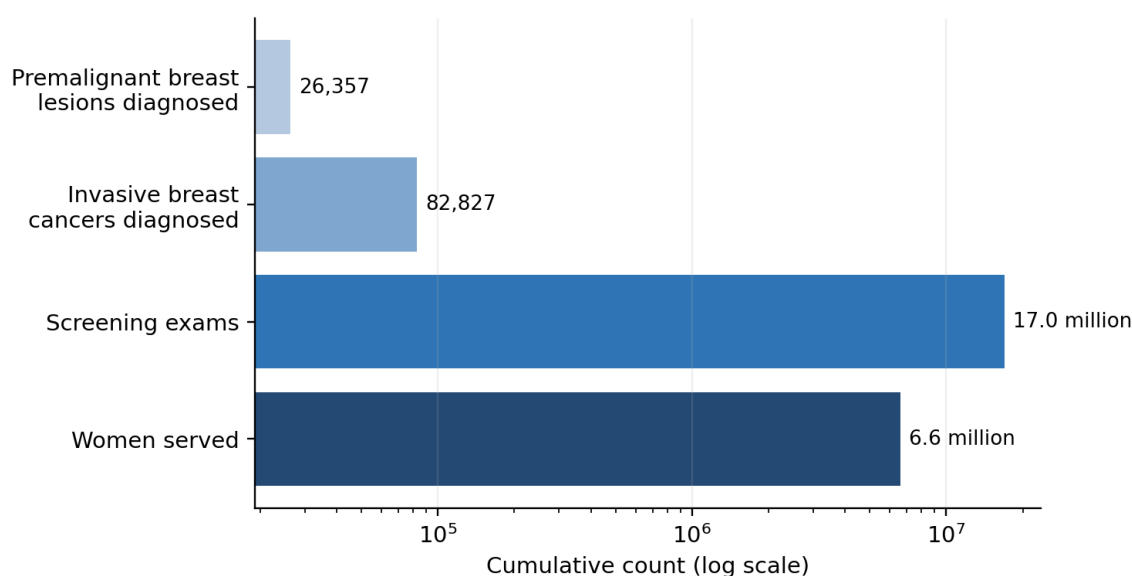


Figure 1. Cumulative reach and diagnoses reported by the National Breast and Cervical Cancer Early Detection Program

Source: Centers for Disease Control and Prevention [3]. Counts are national program totals since 1991 and are displayed on a logarithmic scale because the measures differ substantially in magnitude.

3. Evidence findings

3.1 The scale of safety-net need exceeds current program reach

The NBCCEDP provides a national benchmark for organized breast and cervical cancer screening among low-income and underinsured populations. Since 1991, the program has served more than 6.6 million women and delivered more than 17.0 million screening examinations. It has diagnosed 82,827 invasive breast cancers and 26,357 premalignant breast lesions. These totals demonstrate the value of a structured pathway that includes screening, diagnostic services, and treatment access rather than screening outreach alone [3].

The same data reveal a capacity problem. CDC estimated that 2.6 million U.S. women were eligible for NBCCEDP breast screening during 2020-2021, while the program served 13.5% of that eligible population. California's community programs therefore operate within a larger structural constraint: navigation can improve completion among people reached, but insufficient screening and diagnostic capacity will still leave many eligible people outside the program [3].

Table 1. NBCCEDP source-reported program indicators

Program indicator	Reported value	What it shows for community intervention design
Women served since 1991	More than 6.6 million	Organized public programs can sustain large-scale reach over time.
Breast and cervical screening examinations	More than 17.0 million	Repeated services and follow-up are central to program operation.
Invasive breast cancers diagnosed	82,827	Screening programs must be connected to diagnostic and treatment systems.
Premalignant breast lesions diagnosed	26,357	Programs generate ongoing surveillance and clinical management needs.
Estimated eligible women served, 2020-2021	13.5% of 2.6 million	Funded capacity remains substantially below estimated national need.

Source: CDC National Breast and Cervical Cancer Early Detection Program [3]. Values are national.

3.2 Navigation changes the work required of the program

Educational outreach asks whether a person understands screening. Navigation asks what must happen for the person to complete it. That shift changes program operations. A navigator needs access to appointment information, referral pathways, language services, transportation resources, and a method for tracking unresolved abnormal results. Without those tools, the worker can identify barriers but cannot reliably remove them.

Systematic and meta-analytic evidence indicates that patient navigation increases cancer screening among populations affected by health disparities. Navigation interventions vary, but common effective functions include individualized barrier assessment, appointment scheduling, reminders, coordination across facilities, and sustained follow-up [6]. These functions are especially relevant where community members receive primary care in one system and imaging or biopsy in another.

3.3 Cultural tailoring is an intervention mechanism, not decoration

Language-concordant materials are necessary but not sufficient. Community workers contribute knowledge about how cancer is discussed within families, how fear and stigma shape decisions, which institutions are trusted, and what practical constraints are likely to prevent completion. Programs can use that knowledge to adapt the mode, timing, and content of outreach while preserving the clinical purpose of the intervention.

The California-developed Nuevo Amanecer program is an important example from breast cancer survivorship. The community-based, peer-delivered stress-management intervention was designed for Spanish-speaking Latina survivors in urban and rural settings and reported improved quality of life together with reduced depressive and somatic symptoms. The study illustrates that peers can deliver structured, evidence-informed content with fidelity when training, materials, supervision, and cultural adaptation are built into the model [11].

3.4 The value of continuity accumulates over time

Trust is not a one-time exposure. Community workers become more effective as households, clinics, and referral partners learn that they will remain available. Turnover interrupts relationships and forces programs to rebuild local knowledge. It also weakens accountability for unresolved results because responsibility may change while a patient is moving through the diagnostic pathway.

Continuity requires more than salary support. It requires a defined scope of practice, manageable caseloads, clinical supervision, privacy-compliant access to status information, escalation pathways for urgent results, and support for the emotional demands of cancer navigation. These are standard infrastructure needs, not optional enhancements.

Table 2. Functional model for promotoras and community navigation programs

Function	What the worker does	Required organizational support	Outcome to monitor
Community reach and trust	Conducts language-concordant outreach through trusted local settings and networks	Community partnerships; compensated outreach time; locally adapted materials	Eligible people reached; contact-to-scheduling conversion
Barrier assessment	Identifies transportation, cost, work, childcare, language, disability, and fear-related barriers	Standard assessment tool; resource directory; flexible assistance funds	Barriers identified and resolved
Screening navigation	Schedules mammography, sends reminders, assists with preparation and arrival	Direct scheduling access; imaging agreements; transportation support	Mammography completion; no-show rate
Diagnostic navigation	Tracks abnormal results and coordinates diagnostic imaging or biopsy	Closed-loop registry; named clinical escalation contact; referral authorization support	Diagnostic resolution; days to resolution
Treatment linkage	Connects confirmed cases to oncology and financial or social support	Cancer-center referral agreements; insurance enrollment support	Treatment consultation and initiation
Survivorship support	Delivers structured self-management and psychosocial support; links to survivorship care	Evidence-based curriculum; supervision; referral for clinical symptoms or distress	Quality of life, symptom burden, unmet needs

Functions may be distributed across promotoras, community health workers, patient navigators, nurses, and social workers. The essential requirement is explicit responsibility and handoff.

4. California implementation considerations

4.1 Rural and agricultural communities

Distance, limited imaging capacity, seasonal work, and transportation are often dominant barriers. Community programs should pair outreach with mobile or scheduled regional imaging days, reserve diagnostic slots before screening events occur, and provide navigation that continues after the mobile unit leaves. Evening and weekend scheduling may be more important than additional educational sessions.

4.2 Urban safety-net communities

Urban proximity does not guarantee access. Long wait times, fragmented systems, unstable insurance, language barriers, and competing work or caregiving responsibilities can produce substantial loss to follow-up. Programs should embed navigators within referral workflows and allow real-time scheduling rather than handing patients a phone number for another organization.

4.3 Immigrant and limited-English-proficient communities

Language access must extend beyond the initial outreach encounter to radiology instructions, result notification, diagnostic consent, and oncology consultations. Programs should not rely on family members to interpret high-stakes clinical information. Promotoras can prepare and support patients, but qualified interpretation remains a health-system responsibility.

4.4 Survivorship

Community intervention should not stop at treatment initiation. Survivors may experience persistent pain, fatigue, menopausal symptoms, fear of recurrence, depression, financial toxicity, and difficulty understanding surveillance

recommendations. The Nuevo Amanecer evidence supports structured peer-delivered approaches as one part of a survivorship system, with clear referral thresholds for clinical and mental-health care [11].

5. Recommendations

1. Define promotora and community health worker roles by functions and accountability, not by outreach volume.
2. Provide direct scheduling access and closed-loop status information so community workers can resolve barriers rather than merely identify them.
3. Fund navigation through diagnostic resolution and treatment linkage, not only through mammography completion.
4. Use structured training, competency assessment, supervision, and fidelity monitoring for peer-delivered survivorship interventions.
5. Create formal referral agreements between community organizations, primary-care systems, imaging facilities, diagnostic centers, and oncology programs.
6. Protect workforce continuity through multiyear financing, manageable caseloads, career pathways, and compensation for language and community expertise.
7. Report outcomes stratified by language, race and ethnicity, insurance, rurality, and referral source to determine which populations remain underserved.

6. Limitations

The evidence base includes heterogeneous interventions and job titles, and published studies do not always specify which navigation functions were actually delivered. National NBCCEDP totals demonstrate program scale but do not provide California-specific effect estimates. The Nuevo Amanecer findings apply to a structured survivorship intervention and should not be generalized to all promotora programs without attention to training, content, and implementation fidelity.

7. Conclusion

California's community workforce can reduce breast cancer disparities when it is empowered to do the practical work that fragmented systems otherwise leave to patients. Trust and cultural concordance create entry, but navigation produces completion. The strongest model combines community credibility with clinical integration, closed-loop tracking, and stable financing across screening, diagnosis, treatment, and survivorship.

References

1. White MC, Kavanaugh-Lynch MHE, Davis-Patterson S, Buermeyer N. An Expanded Agenda for the Primary Prevention of Breast Cancer: Charting a Course for the Future. *International Journal of Environmental Research and Public Health*. 2020;17(3):714. doi:10.3390/ijerph17030714.
2. U.S. Preventive Services Task Force. Screening for Breast Cancer: U.S. Preventive Services Task Force Recommendation Statement. *JAMA*. 2024;331(22):1918-1930.
3. Centers for Disease Control and Prevention. National Breast and Cervical Cancer Early Detection Program: About the Program. Updated February 20, 2026. Accessed July 2026.
4. Sabatino SA, Lawrence B, Elder R, et al. Effectiveness of interventions to increase screening for breast, cervical, and colorectal cancers: nine updated systematic reviews for the Guide to Community Preventive Services. *American Journal of Preventive Medicine*. 2012;43(1):97-118.
5. Baron RC, Rimer BK, Breslow RA, et al. Client-directed interventions to increase community demand for breast, cervical, and colorectal cancer screening: a systematic review. *American Journal of Preventive Medicine*. 2008;35(1 Suppl):S34-S55.
6. Nelson HD, Cantor A, Wagner J, Jungbauer R, Fu R. Effectiveness of patient navigation to increase cancer screening in populations adversely affected by health disparities: a meta-analysis. *Journal of General Internal Medicine*. 2020;35(10):3026-3035.
7. Freeman HP, Rodriguez RL. History and principles of patient navigation. *Cancer*. 2011;117(15 Suppl):3539-3542.
8. California Department of Public Health. California Comprehensive Cancer Control Plan, 2022-2026. Sacramento, California.
9. Glasgow RE, Vogt TM, Boles SM. Evaluating the public health impact of health promotion interventions: the RE-AIM framework. *American Journal of Public Health*. 1999;89(9):1322-1327.
10. Proctor E, Silmere H, Raghavan R, et al. Outcomes for implementation research: conceptual distinctions, measurement challenges, and research agenda. *Administration and Policy in Mental Health*. 2011;38(2):65-76.
11. Nápoles AM, Santoyo-Olsson J, Stewart AL, et al. Randomized controlled trial of a community-based, peer-delivered stress management intervention to improve quality of life of Latinas with breast cancer. *American Journal of Public Health*. 2015.
12. Wells KJ, Battaglia TA, Dudley DJ, et al. Patient navigation: state of the art or is it science? *Cancer*. 2008;113(8):1999-2010.
13. Rosenthal EL, Brownstein JN, Rush CH, et al. Community health workers: part of the solution. *Health Affairs*. 2010;29(7):1338-1342.